

3. Assisted Living Facilities / Nursing Homes

ICP Summary

Assisted living facilities and nursing homes are a high-fit vertical when scoped around documentation, family communication, census/occupancy tracking, appointment coordination, transportation coordination, admissions/discharges, and compliance form tracking. The Virtual Assisted Living Coordinator and Virtual Nursing Home Assistant JDs map directly to these workflows. The VALC role supports resident documentation, family communication logs, appointments, transportation, consent forms, and care-plan document tracking. The VNHA role supports census logs, admission/discharge documentation, bed availability, compliance form due dates, and audit-readiness documentation.

Best-Fit Senior-Care Segments

ICP Tier	Facility Type	Facility Profile	Fit Level	Recommended Motion
ALF A	Small assisted living facility	6–50 beds, owner/operator-led	Selective	Single-seat admin coordinator
ALF B	Mid-sized assisted living / memory care	50–150 beds	High	VALC or compliance support
ALF C	Multi-site ALF operator	2–10 facilities	Very High	Function-specific team
NH A	Independent nursing home	80–200 beds	High	VNHA or admissions/compliance support
NH B	Multi-facility nursing home operator	2+ facilities	High but longer cycle	Structured team
Senior Living D	Large corporate senior living group	Regional or national operator	Opportunistic	Account-based, governance-heavy

Ideal Customer Profile — Assisted Living

Primary decision-maker: Owner/operator, administrator, executive director, resident care director, business office manager.

Operational pain: Missing documentation, family update burden, appointment tracking, transportation coordination, resident status-change logs, admission packets, discharge packets, compliance reminders, staff overwhelmed by admin tasks.

Remote readiness: Uses EHR/LTC software, shared drive, census dashboard, facility management software, or digital documentation process.

Budget maturity: Can justify admin support as compliance protection, family communication support, and staff time relief.

Primary buyer psychology: Wants operational relief without risking resident care quality or survey readiness.

Ideal Customer Profile — Nursing Homes

Primary decision-maker: Administrator, director of nursing for workflow input, business office manager, regional operations leader.

Operational pain: Census accuracy, admissions/discharges, compliance form tracking, physician appointment coordination, family meeting documentation, audit preparation, document naming/filing, recurring regulatory deadline tracking.

Remote readiness: Uses PointClickCare, MatrixCare, Netsmart, American HealthTech, PCC, shared census reports, or structured document repositories.

Budget maturity: More formal than ALF; needs scope, SLA, reporting, and clear non-clinical boundaries.

Primary buyer psychology: Wants reliability, survey readiness, and documented follow-through.

Role Mapping

H& Jobs Role	Senior-Care Use Case	Fit
Virtual Assisted Living Coordinator	Resident admin records, family communication logs, appointment/transport coordination, consent and POA form tracking	Primary for ALF
Virtual Nursing Home Assistant	Census, occupancy, admissions/discharges, compliance logs, audit readiness	Primary for nursing homes
Virtual Medical Scheduler / Receptionist Adaptation	Appointment scheduling, family meeting scheduling, transportation calendar support	Secondary

H& Jobs Role	Senior-Care Use Case	Fit
HR & Payroll Assistant	Employee files, onboarding checklists, time-off logs, payroll prep	High for multi-site or high-turnover facilities
Executive Assistant	Executive director support, meeting notes, vendor coordination, board/owner communications	Optional
Medical Billing Coordinator	Facility AR/billing support only if narrow, trained, and separated from clinical documentation	Conditional

The HR & Payroll Assistant is relevant because the JD already covers employee records, onboarding/offboarding documentation, payroll input preparation, leave tracking, and HRIS/spreadsheet workflows.

High-Intent Buying Triggers

Trigger	Interpretation
"Our staff spends too much time updating families"	VALC fit
"Admissions packets are incomplete"	VNHA / compliance admin fit
"We need better census tracking"	VNHA fit
"Transportation and appointments are hard to coordinate"	VALC fit
"Survey prep is stressful"	Compliance documentation support fit
"We have several facilities but inconsistent records"	Multi-site structured team opportunity

Senior-Care Disqualification Rules

Disqualify or defer facilities that:

- Want remote staff to perform clinical triage, nursing tasks, resident assessments, care-plan authoring, or medication-related decisions.
- Do not have a facility-side owner for escalation.
- Use verbal-only processes and refuse written workflows.
- Cannot provide secure access to documentation systems or approved communication channels.
- Expect remote staff to replace on-site resident-facing caregivers.

- Have active severe compliance issues and want H& Jobs to “clean it up” without leadership ownership.

Florida Launch Priority — Senior Care

Highest priority regions:

1. Southwest Florida: Lee, Collier, Sarasota, Manatee
2. Tampa Bay: Hillsborough, Pinellas, Pasco
3. Central Florida: Orange, Seminole, Osceola, Lake
4. South Florida: Palm Beach, Broward, Miami-Dade
5. North Florida: Duval, St. Johns, Clay